

RESEARCH ARTICLE

Prevention of Violence Against Children: A Qualitative Exploration of Colombian Policymakers', Programme Facilitators' and Caregivers' Perceptions

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ABSTRACT

Objective: Violence against children (VAC) is a global public health and human rights issue that can lead to long-lasting negative consequences for individual and societal outcomes. While extensive evidence indicates that parenting programmes might be effective in preventing VAC, there are several unsolved questions on how to ensure interventions are acceptable, feasible, effective and sustainable, particularly in low- and middle-income countries (LMICs).

Method: In this study, we report findings from a qualitative examination of policymakers' ($N=10$), early childhood and parenting programme facilitators' ($N=20$) and parents' and other caregivers' ($N=38$) perspectives on VAC prevention to examine the implementation ecosystem of parenting programmes in Colombia, including contextual risk and protection factors, features of existing programmes, and stakeholders' needs. We conducted interviews and focus groups using a semistructured format, along with a thematic approach, to analyse the data from each group of participants (i.e., policymakers, facilitators and caregivers) independently.

Results: Overall, the data revealed the critical role of intersecting and interacting factors at the micro (e.g., caregivers' capabilities and beliefs), meso (e.g., programme content and delivery approaches) and macro (e.g., policymakers' vision and existing infrastructure) levels in exacerbating risks/imposing barriers versus protecting/promoting VAC prevention.

Conclusions: These findings provide evidence on the implementation ecosystem of prevention programmes to inform the design of novel strategies and programmes aimed at preventing violence and promoting families' well-being and young children's development.

1 | Introduction

Violence against children (VAC) is a significant human rights and public health issue that obstructs human development and societal well-being. While millions of children younger than five living in low- and middle-income countries (LMICs) already experienced physical and psychological violence in their homes before COVID (Hillis et al. 2016), growing evidence indicates that the pandemic and the growing climate crisis have intensified VAC (Bhatia et al. 2021; Cuartas et al. 2023). The high prevalence of VAC leads to substantial economic and societal costs, as physical and psychological forms of VAC early in life can weaken the developing brain and other biological systems with long-term consequences on physical and mental health, learning, behaviour and well-being (McLaughlin, Weissman, and Bitrán 2019; Shonkoff and Garner 2011).

Systematic reviews and meta-analyses show that parenting programmes have been effective in reducing VAC and key risk factors for VAC (e.g., related to knowledge and attitudes, parenting stress and family environment) with estimated effect sizes ranging from $d = 0.21$ to $d = 0.42$ SD (Chen and Chan 2015; Coore Desai, Reece, and Shakespeare-Pellington 2017; Knerr, Gardner, and Cluver 2013; McCoy, Melendez-Torres, and Gardner 2020). Parenting programmes are flexible and have been implemented using multiple delivery platforms, including home visits, group meetings and even via SMSs and online platforms (Lansford et al. 2022; UNICEF 2020). Despite their effectiveness, few parenting programmes have been scaled up, and there are several unsolved questions on how to scale up evidence-based interventions successfully (Aboud, Yousafzai, and Nores 2018; Britto et al. 2022; Shenderovich et al. 2021).

The implementation and scalability of parenting programmes depend on the whole implementation ecosystem, including interactions between 'supply'- and 'demand'-side factors at multiple levels of the ecology of human development (Britto et al. 2022; Shenderovich et al. 2021). The supply side refers to what the programme offers, including content and delivery approaches. The demand side comprises contextual factors that underlie the acceptability and perceived utility of programmes, including factors at the society (e.g., cultural influences, social norms, policy and service infrastructure), community (e.g., contextual resources and barriers, facilitators' skills and training, facilitators' social norms and beliefs about physical punishment) and family and individual levels (e.g., beneficiaries' needs, beliefs, self-efficacy and mental health) (Britto et al. 2022). While there has been extensive research on the design of content and delivery approaches for parenting programmes (i.e., supply side), historically, there is less evidence on the demand side, which constitutes a barrier to the sustainability and scaling up of effective programmes (Bornstein et al. 2022; Britto et al. 2022; Lansford et al. 2022). For example, financial constraints may make it difficult for some families to pay for transportation to attend programme sessions, undermining implementation outcomes and scalability. Similarly, social norms and beneficiaries' beliefs about the efficacy of a programme may increase their willingness to attend the programme's sessions, increasing positive implementation outcomes.

Evidence on the whole implementation ecosystem of parenting programmes is particularly scarce in LMICs, which are heterogeneous settings vastly underrepresented in developmental and prevention science (Coore Desai, Reece, and Shakespeare-Pellington 2017; Draper et al. 2023). LMICs are widely diverse in culture (e.g., language, norms and family composition), socio-economic dynamics (e.g., resources) and social infrastructure (e.g., systems and programmes and human resources) and often face significant adversity and resource constraints (Aboud, Yousafzai, and Nores 2018). Collectively, these factors may affect the implementation and scalability of parenting programmes, even when they are shown to be effective in reducing VAC or risk factors for VAC (Britto et al. 2022; Leijten et al. 2016). Therefore, there is a need for further research on the implementation ecosystem of parenting programmes in LMICs, mainly to understand contextual affordances.

In this study, we report findings from a qualitative examination of policymakers', early childhood development (ECD) parenting programme facilitators' and parents' and other caregivers' perspectives on violence prevention to delve into 'demand'- and 'supply'-side factors of a parenting programme in Colombia. In this country, approximately 10% of children have been internally displaced due to the civil conflict (Red Nacional de Información 2018) and at least 1.7 million children younger than five were physically maltreated before COVID, with the highest rates in regions more affected by the civil conflict (Cuartas 2018). To date, there are no scaled-up universal violence prevention parenting programmes in Colombia (Cuartas et al. 2022), despite evidence from small-scale studies that have demonstrated effectiveness (e.g., Skar et al. 2021).

Within this context, in 2021, the Colombian National Government launched a national plan to reduce VAC, which called for 'creating, strengthening, or designing services, programs, and strategies aimed at preventing physical punishment and cruel, humiliating, or degrading treatment of children' (author's translation; Gobierno de Colombia 2021, p. 51). To respond to these policy calls, recent research has examined the supply side of parenting programmes in Colombia (e.g., core content and delivery approaches) and some components of the demand side, including existing social infrastructure and services in which a violence prevention strategy could be embedded (Cuartas et al. 2022). Yet, little is known about other critical aspects of the implementation ecosystem.

For policymakers, the study aimed to examine their perspectives on existing social and service infrastructure, barriers and facilitators to implementation and pathways to scale for violence prevention parenting programmes. For programme facilitators, the goal was to explore their perspectives on contextual and family factors contributing to VAC, features of current programmes aimed at reducing VAC and challenges and needs for delivering VAC prevention programmes. Finally, for parents and caregivers, the study aimed to explore beliefs about physical punishment, perceptions of common behaviour problems of children, child behaviour management strategies used and daily routines and interactions at home.

2 | Methods

We conducted this study between February and November 2022. The study received ethical approval from the Institutional Review Board (IRB) at Harvard University on November 19, 2021, and received technical approval from the Instituto Colombiano de Bienestar Familiar (ICBF) on January 25, 2022.

2.1 | Participants

We employed different sampling procedures for policymakers, programme facilitators and parents and caregivers. First, we worked with staff from the ICBF Early Childhood division to identify key policymakers working on ECD, parenting, and/or violence prevention policies and/or programmes in Colombia. We identified and invited 10 policymakers to take part of the study, and all agreed to participate. Sampled policymakers came from the Alianza Nacional contra la Violencia hacia Niños y Niñas (National Alliance to End Violence Against Children) ($n = 1$), the ICBF Early Childhood division ($n = 3$), the ICBF Protection division ($n = 1$), the ICBF Families and Communities division ($n = 1$), the ICBF Bogotá regional office ($n = 1$), the Ministry of Health ($n = 2$) and the Ministry of Education ($n = 1$). Sampled policymakers were mostly female ($n = 9$), between 33 and 60 years old, and most ($n = 7$) had master's degrees as maximum education level (see Table 1 for details).

We selected programme facilitators in collaboration with the ICBF. First, we selected the largest urban municipality (i.e., capital) and the rural municipality with the highest rate of VAC for each of the three departments (i.e., largest administrative division) in Colombia (i.e., Cundinamarca, Antioquia and Atlántico). Subsequently, we selected ECD service units (UDS, for its acronym in Spanish) from three different ICBF service modalities, i.e., (1) institutional or centre based, (2) family or home based and (3) community or community based (for further details on services modalities, see Cuartas et al. 2022), for all six municipalities, with the exception of one where there were no family-based ECD service units. Then, we worked with ICBF's regional directors and technical assistance officers of the ECD division to identify programme facilitators who worked with families. The ICBF contacted the selected service units and requested for consent to contact the facilitators. The ICBF then provided us with a list of contact details for the facilitators. We randomly contacted

44 facilitators, ensuring that there was at least one facilitator per modality in each municipality. Out of the 44 facilitators initially contacted, 20 who responded and expressed interest were selected to participate in the interviews. Programme facilitators were ECD teachers ($n = 6$), psychosocial professionals ($n = 7$) and Madres Comunitarias (i.e., community mothers) ($n = 7$). Of the 20 facilitators, 19 were female. Participants were between 22 and 54 years, and 52% had secondary education.

Regarding parents and caregivers, we selected three localities in Bogotá with the highest rates of child maltreatment, which collectively account for about 41.4% of all reported cases in 2021, according to the Secretaría de Salud (Department of Health) of Bogotá. We worked with the ICBF to select two or three ECD service units (UDS) in each locality, sampling the UDS of three different service modalities (i.e., institutional, family and community modalities). We invited families of young children in the selected UDS through a Qualtrics form, which families used to enrol in the study if they wished to. A total of 473 caregivers filled out the Qualtrics form, expressing interest in participating. We randomly selected a final sample of 38 parents and caregivers, most of them female ($n = 37$), aged 18–45 years old, with one or two children younger than five and most with secondary education ($n = 28$).

2.2 | Procedure

We conducted interviews with policymakers ($n = 10$), programme facilitators ($n = 20$), parents and caregivers ($n = 20$) and two focus groups with parents and caregivers ($n = 18$). We conducted the interviews and/or focus groups after obtaining informed consent. J.C. conducted the interviews with policymakers and L.B. with facilitators and caregivers, and L.B. moderated the focus groups, all in Spanish. Interviews with policymakers lasted between 40 and 60 min, with facilitators between 15 and 40 min, and with caregivers between 15 and 35 min, whereas the focus groups lasted approximately 60–90 min. All interviews were conducted using Zoom or WhatsApp, and focus groups were conducted in person at local community centres.

Interviews and focus groups used a semistructured format following different guiding questions to facilitate discussion with each group of participants. The semistructured format

TABLE 1 | Characteristics of the participants.

	Policymakers ($n = 10$)	Facilitators ($n = 20$)	Caregivers and parents ($n = 38$)
Age, M (range)	45.6 [33–60]	36.1 [22–54]	27.8 [18–45]
Gender, n female (%)	9 (90.0%)	19 (95%)	37 (97.4%)
Education, n (%)			
Less than secondary complete	0 (0%)	3 (15%)	10 (26%)
Secondary complete	0 (0%)	2 (10%)	20 (52%)
Undergraduate education	1 (10%)	15 (75%)	8 (21%)
Graduate education	9 (90%)	0 (0%)	0 (0%)

Note: M = mean; n = total number.

allowed us to hold organic conversations and ask follow-up questions to dive deeper into any topic of interest. All interviews and focus groups included opening questions about participants' characteristics, open-ended introductory questions to get to know the participant and build interest to encourage open sharing, several key questions on the study's main topics and an ending question (see Table 2 for example questions). Given that VAC is a sensitive topic, we decided to conduct both interviews and focus groups with parents and caregivers to triangulate and obtain more comprehensive data (Guest et al. 2017; Lambert and Loiselle 2008).

2.3 | Analysis

All interviews and focus groups were audio recorded and transcribed verbatim, and the accuracy of transcriptions was assessed against the audio recording. We used a thematic approach (Braun and Clarke 2006; Ritchie and Spencer 2002) to analyse the data from each group of participants independently. We began by examining the transcripts to understand the data. Subsequently, we considered main topics from the interview guide and study's research questions and used an open-coding approach to identify emergent codes from the data of each group of participants, which we grouped into larger themes using axial coding. When segments of text comprised more than one code, we applied all relevant codes. J.C. and L.B. reviewed, discussed and decided the final set of themes. We organized the themes in separate documents for policymakers, facilitators, and parents and caregivers and selected representative quotes for each theme. Finally, we

compared the themes across the different groups to synthesize key themes across stakeholders. Participants' quotations were translated from Spanish to English by J.C. and L.B. All analyses were conducted using NVivo 1.6.1.

3 | Results

Table 3 presents the themes, codes and subcodes that emerged from the data.

3.1 | Policymakers

Four overarching themes that emerged from policymakers' data are as follows: (1) drivers of VAC, (2) promotive factors that facilitate the prevention of VAC, (3) barriers to preventing VAC and (4) existing social and service infrastructure to prevent VAC in Colombia.

3.1.1 | Drivers of VAC

Policymakers' perceive structural factors (e.g., poverty and discrimination), gender-based violence, contextual violence (e.g., the civil conflict), caregivers' own experiences of violent punishment in childhood (i.e., intergenerational transmission of parenting practices and violence), social norms and beliefs that legitimize violence and caregiver stress and daily stressors as the main drivers of VAC in Colombia. For example, one policymaker expressed that some drivers of VAC 'are associated

TABLE 2 | Examples of questions to facilitate discussion in the interviews and focus groups.

	Policymakers	Facilitators	Caregivers and parents
Introductory questions	Tell me about your current work at (institution)?	- Tell me a little bit about your current work with the ICBF? - What do you do with families? - What are the goals/aims of your work with families?	- Think about being a parent and what things you enjoy the most and what is more difficult. - Let's start by talking about the things you enjoy. - Now tell me about some things that are more difficult
Key questions	- What are some of the programmes and strategies aimed at prevention violence against children that the government is currently implementing? - Which have been the main enablers and barriers to implementing violence prevention parenting programmes in Colombia?	- Do you include any VAC prevention in your current work? - Have you received any training from the ICBF? Please tell us about such experience - What type of support do they think parents need to reduce VAC?	- Can you give me some examples of your child's behaviours that you found irritating and could lead you to lose control and scream or spank him/her? - Sometimes, children behave good and sometimes they give a lot of trouble. Think about the times your child doesn't behave so well. What sort of things does he or she do and say?
Ending question	Do you have any final questions for me?	Do you have any final questions for me?	Do you have any final questions for me?

TABLE 3 | Themes, codes and subcodes.

Policy-makers	Facilitators	Parents and caregivers	
Drivers of VAC • Structural factors ◦ Poverty ◦ Discrimination • Gender-based violence • Contextual violence • Caregivers' violent childhood experiences • Attitudes and beliefs • Stress and daily stressors Promotive factors that facilitate the prevention of VAC • Adapting programmes to the culture and local reality • Communicate contents in nonstigmatizing ways • Innovation • Flexibility • Participatory design processes • Articulating programmes into existing infrastructure Barriers to preventing VAC • Lack of shared vision between policymakers • Lack of monitoring and evaluation • Challenges to reach families • Programme targeting • Challenges in sustainability • Inadequate training for programme facilitators • Social norms Existing infrastructure to prevent VAC • National Alliance to Prevent VAC • Services in the ICBF ◦ Early childhood division ◦ Communities and Families division • Training–ICETEX	Drivers of VAC • Emotions ◦ Lack of patience ◦ Stress • Cognitions ◦ Lack of knowledge on parenting and discipline ◦ Inadequate discipline • Cultural and contextual factors ◦ Parents' childhood experiences ◦ Social norms Caregivers' needs • Communication skills • Empathy • Limit setting • Self-regulation • Knowledge on child development • Self-awareness and healing past traumas Facilitators' needs • Education and training • Receptivity from parents • Material resources • Tools and strategies about self-regulation and childrearing Current challenges • With the ICBF ◦ Ineffective training ◦ Too much administrative work ◦ Perceived lack of support ◦ Lack of material resources • With parents ◦ Lack of consistency ◦ Little interest ◦ Parents are too busy ◦ Negative perceptions of the ICBF ◦ Social norms ◦ Presence of multiple caregivers who do not receive services • Contextual ◦ Number of 'dysfunctional families' ◦ Teenage parents ◦ Parents' economic challenges ◦ Intimate partner violence ◦ Contextual violence Delivery approaches for programmes • Effective ◦ Offering spaces for emotional communication and stress relief ◦ Demonstration and modelling ◦ Role-playing ◦ Audiovisual resources ◦ Use of WhatsApp ◦ Media campaigns • Noneffective ◦ Providing informational materials only	Children's behaviours • Desired behaviours ◦ Compliant ◦ Behaving as parents expected ◦ Helpful and caring ◦ Positive attitude ◦ Avoiding 'bad behaviours' (i.e., behaviours which were difficult to manage for the parents). • Undesired behaviours ◦ Aggressiveness ◦ Disorganized ◦ Feisty or rude ◦ Noncompliance ◦ Excessive crying ◦ Hyperactivity ◦ Tantrums Parenting strategies • Behaviour management ◦ Calming them down ◦ Distracting them ◦ Talking/reasoning ◦ Removing privileges ◦ Scolding them ◦ Physical punishment ◦ Reinforcement and promotion of positive behaviours ◦ Showing affection ◦ Praising ◦ Rewards ◦ Explaining things ◦ Leading by example Parenting goals • Discipline ◦ Avoid spoiling children ◦ Increase compliance ◦ Raise children to be 'good people' ◦ Ensure respect • Learning ◦ Difference between rights and wrong ◦ Values ◦ Competencies ◦ Avoid danger Multiple caregivers • Grandmothers • Caregivers' siblings • Other adults in the community Sources of information about parenting • Childhood experiences • Past experiences • Advice from family • The ICBF	Drivers of VAC • Emotions ◦ Stress ◦ Anger ◦ Lack of patience • Cognitions ◦ Perceived lack of effectiveness of other discipline strategies • Cultural and contextual factors ◦ Multiple responsibilities ◦ Lack of communication ◦ Lack of support ◦ Child age • Beliefs about childrearing Protective factors for VAC • Self-management ◦ Self-regulation ◦ Patience ◦ Knowledge on children's needs ◦ Organizational skills • Things parents enjoy ◦ Spending quality time with children • Negative effects of physical punishment ◦ Increases noncompliance ◦ Causes fear • Childhood experiences

to broader cultural ideas, collective imaginaries, and practices embedded within our society, for example, systems like the patriarchy'. Another policymaker stated that VAC is 'related

to mental health and emotion regulation', which is challenging 'in a country that does not want to speak about mental health'.

3.1.2 | Promotive Factors

Policymakers think that adapting programmes to meet the culture and local realities of communities and communicating content in nonstigmatizing ways are key factors that facilitate the implementation and success of violence prevention strategies. Policymakers also consider innovation and flexibility to be key promotive factors. For instance, a policymaker mentioned that to ensure feasibility, a programme in Colombia

[...] has to be very flexible, what I mean is that it has to have a high level of flexibility with some minimums and some immovable essentials [...] almost like pieces, no? [...] like modular? so in some cases we can use this module, or this part works here, but this other part does not.

In addition, they identified participatory design processes with policymakers, programme facilitators and families as a key facilitator for the design and implementation of violence prevention programmes, as well as integrating prevention programmes into existing social and service infrastructure to facilitate implementation and scalability.

3.1.3 | Barriers

Policymakers think that the main barriers for the success of violence prevention strategies have been the lack of shared vision between different policymakers or, as stated by a policymaker, difficulties 'in reaching technical consensus', lack of monitoring and evaluation systems that impede understanding whether programmes are effective and challenges that the ICBF faces when trying to reach families, as stated by a participant:

we at the national level have to go through the regional and zonal levels to reach the program facilitators and the staff, who are ultimately the ones who have direct contact with children and their families, so I believe that in this chain [...] things get lost in the different links that exist.

Other barriers include problems in programme targeting (e.g., most programmes target families with children older than 2 years), challenges in programme sustainability, inadequate training for programme facilitators and social norms that legitimize violence, as explained by one policymaker:

program facilitators or in general the agents who are in contact with families and children are not always as convinced of the need to eliminate physical punishment in parenting practices, so I think that is still a barrier.

3.1.3.1 | Existing Infrastructure. Finally, policymakers identified several existing programmes and services that could serve as avenues for the implementation at scale of violence

prevention strategies, including the national alliance to prevent VAC, services in the ICBF divisions of early childhood and families and communities, nutrition services and use of existing training mechanisms for programme facilitators (e.g., ICETEX fund).

3.2 | Programme Facilitators

We identified five themes from the facilitators' data: (1) drivers of VAC, (2) caregivers' needs, (3) facilitators' needs, (4) current challenges and (5) delivery approaches for prevention programmes that have been effective.

3.2.1 | Drivers of VAC

Facilitators shared their perceptions about the main drivers of VAC in Colombia. Among these drivers, they mentioned parents' childhood experiences with their own parents (i.e., intergenerational transmission of parenting practices and violence), cultural factors and social norms, lack of knowledge on parenting and discipline (e.g., 'mothers sometimes say they do not have the tools, they do not know what to do'), inadequate discipline practices (e.g., lack of limit setting), lack of patience and emotional distress.

3.2.2 | Caregivers' Needs

Facilitators mentioned some of the needs they perceive parents have regarding childrearing, including a need to learn more about the importance of dialogue and reasoning for childrearing, empathy with their children, limit setting, self-regulation (e.g., stress and anger management) and knowledge of child development, as explained by one facilitator:

I feel that there is a specific lack of knowledge among parents about the importance of this stage [early childhood] in the long term, they consider that children do not know and do not understand anything because they are small.

Another need that parents have, which facilitators identified and reportedly attended through current interventions, relates to emotional self-awareness and healing past traumas. One facilitator referred to some of the workshops they carried out saying: '[...] through play, through the recognition of our own emotions as caregivers, as fathers, as mothers, [parents need to] heal that inner child that is harmed for some of them'.

3.2.3 | Facilitators' Needs

Facilitators also discussed some of the needs they have regarding their work with families, including a need for further education and training, increased receptivity from parents, material resources (e.g., better spaces to meet with families) and more tools and strategies related to emotion regulation and childrearing to share with families.

3.2.4 | Challenges

Facilitators also mentioned some of the challenges they face when working on the prevention of VAC with families. Among these, they discussed challenges related to working with the ICBF, including ineffective training, too much administrative work that limits the time they can devote to families, perceived lack of support and lack of material resources.

They also mentioned challenges with parents, including lack of consistency (i.e., parents do not use the tools and parenting strategies that facilitators suggest), little interest, being too busy, negative perceptions of the ICBF (i.e., parents believe that the ICBF will take away their children), social norms that make parents believe that they can do whatever they like with their children and the presence of multiple caregivers in households who do not receive services or support from the ICBF, as explained by one facilitator:

But in the reality of family dynamics [...] there are additional caregivers, it is very complex (...) yes, yes, we have a lot of children of working parents and generally the children are cared for by grandparents, aunts, or a person X who is not a part of the family, right?

Facilitators also mentioned contextual factors that make their work more challenging, including the number of 'dysfunctional families', teenage parents, families' economic challenges, widespread intimate partner violence and contextual violence, as expressed by one facilitator:

I don't know, violence here is everywhere, there is violence, there is an issue of social violence, economic violence, but on a general level, not only within the home, but in public spaces as well.

3.2.5 | Delivery Approaches for Prevention Programmes

Finally, facilitators shared their ideas on delivery approaches for parenting and prevention programmes that they consider to be effective. Among other delivery approaches, they consider effective offering spaces for emotional communication and stress relief, demonstration and modelling of behaviours and strategies, role-playing and practice, use of audiovisual resources and use of WhatsApp to keep constant communication with families. Furthermore, they consider media campaigns to be effective. Finally, some facilitators mentioned that providing informational materials only for parents to take home is not particularly effective, as parents do not have the time to review them.

3.3 | Parents and Caregivers

Seven themes that emerged from the data from parents and caregivers are as follows: (1) children's behaviours, (2) parenting strategies, (3) parenting goals, (4) drivers of VAC, (5)

protective factors for VAC, (6) the importance of multiple caregivers in childrearing and (7) sources of information for parenting.

3.3.1 | Desired and Undesired Children's Behaviours

Caregivers shared their perceptions of what constitutes children's 'good behaviour' and 'misbehaviour'. On the one hand, caregivers listed as examples of good behaviour being compliant and doing things the way parents expected, being helpful and caring, having a positive attitude and avoiding 'bad behaviours' that irritate caregivers. On the other hand, caregivers considered that 'misbehaviour' or undesirable behaviour includes aggressiveness (e.g., hitting and biting), being disorganized, being feisty or rude, noncompliance, excessive crying, hyperactivity and tantrums.

3.3.2 | Parenting Strategies

The strategies caregivers use to manage children's undesirable behaviours include calming them down (e.g., by hugging or showing affection), distracting them (e.g., using toys), talking and reasoning, withholding privileges (e.g., not allowing play), scolding them and using physical punishment like spanking or hitting with objects, as explained by one caregiver:

I was organizing the pots and she [the child] was throwing them, and then I took her and hit her with the same pot, I didn't hit her hard, but then for them [children] that's something meaningful [...] it was enough, and she didn't take my pots again.

Caregivers also discussed strategies to reinforce desired behaviours and promote children's development, including showing affection, praising (e.g., 'sometimes they make things right and we praise them'), using rewards (e.g., 'taking them for an ice cream'), explaining things and leading by example, as some caregivers considered that 'children are a reflection of their parents'. Caregivers also mentioned some play/learning activities that children enjoy, including playing games (e.g., role-playing), playing with toys, listening to music, and singing.

3.3.3 | Parenting Goals

Caregivers mentioned that they use the abovementioned parenting strategies to avoid spoiling children, to make them compliant and to raise them to be 'good people' and to ensure that children respect their caregivers. Caregivers hope to teach children the difference between right and wrong, values, competencies, and danger avoidance.

3.3.4 | Drivers of VAC

The data showed multiple individual and contextual factors that might increase the risk of VAC. Caregivers discussed that they

use physical punishment when other discipline strategies (e.g., reasoning) are not effective, as well as due to stress, anger and lack of patience. Other factors that might increase the risk of VAC include taking care of the child while having other responsibilities (e.g., a job), lack of communication between caregivers and lack of support from other caregivers (e.g., being a single mother or staying alone in the home with the child). Child age is another potential risk factor for VAC, as mentioned by a participant: 'ah, when they are older, yes! If they do something wrong, one must spank them (laughs)'. Finally, we identified that caregivers' perceptions on the autonomy of parenting choices, even with physical punishment, might be another risk factor: 'everyone can educate their children as they want, right? I don't get involved in that'.

3.3.5 | Protective Factors for VAC

Participants mentioned several factors that may reduce the risk of VAC, including having self-management and organizational skills, knowing their children's needs and having patience and emotional self-regulation, as stated by a caregiver:

[...] you have to [...] breathe, [...] calm down, for example when you say you can't take it anymore, because let's say it's not their [children's] fault that you are stressed, that you are tired.

Spending quality time with children (e.g., playing) is another factor that may prevent VAC. Some parents believed that violent punishment is not necessary in early childhood because young children 'do not understand' and it is better to be patient, whereas other parents believe the opposite, i.e., violent punishment is not warranted because children understand so it is better to reason with them. Finally, another protective factor might include caregivers' own childhood experiences, including having a caring parent and, for some parents, even having experienced physical punishment in childhood, as illustrated in the following except:

I had a very difficult upbringing, because my mother left us, and my father raised us, and my father used to hit us for everything, he yelled at us, he used to say bad words, so I don't want that for my daughter.

3.3.6 | Multiple Caregivers

Another theme that emerged from the interviews and focus groups was the importance of extended networks of caregivers and multiple caregivers, including grandmothers, caregivers' siblings and other adults (like child minders from their communities).

3.3.7 | Sources of Information About Parenting

Finally, caregivers mentioned that their main sources of information about parenting include their own childhood experiences, their past experiences minding for children (e.g.,

taking care of other babies in their families), advice from their families and programmes, workshops, and information from the ICBF.

3.4 | Integrating Stakeholders' Perspectives

Policymakers, practitioners and caregivers identified similar and complementary sets of risk (barriers) and protective (promotive) factors for preventing VAC (Table 4). These sources of risk and protection were identified at the family level, at the programmes and facilitators level, and at the social structures and policies level. The most salient themes are related to developmental aspects (e.g., children's age and behaviours), caregivers' emotional (e.g., self-regulation and well-being), cognitive (e.g., knowledge, attitudes and beliefs), contextual (e.g., poverty, violence, stressors and caregivers' upbringing) and relational characteristics (e.g., family composition), as well as to the importance of interactions between factors at different levels of the ecology. There were also consistent mentions to the need for more flexibility and innovation in programme design and implementation. One theme with somewhat contradictory perspectives was the role of caregivers' upbringing, as some stakeholders considered that having experienced violence in their childhood increased the risk for intergenerational transmission. In contrast, others thought that such experiences reduced the risk as caregivers did not want to replicate such violence.

4 | Discussion

This study examined Colombian policymakers', programme facilitators' and caregivers' perceptions of the drivers of VAC, characteristics of existing ECD and parenting programmes and other contextual affordances to examine the implementation ecosystem of prevention programmes in Colombia. Overall, the qualitative data revealed the critical role of intersecting and interacting factors at the micro (i.e., family), meso (i.e., programme and facilitators) and macro (policy and social structures) levels in exacerbating risks/imposing barriers versus protecting/promoting VAC prevention (see Table 4 and Figure 1), consistent with a social-ecological perspective (Bronfenbrenner and Morris 2007; World Health Organization 2016).

At the micro level, the study participants identified developmental, cognitive, emotional, relational and contextual factors as key factors that drive or prevent VAC. These findings align with developmental theories highlighting the dynamic and transactional nature of development and parenting or the changing needs and challenges that children and caregivers face over time (Sameroff 2010). The results are also consistent with contemporary perspectives on the critical role of parents' cognitive (e.g., knowledge) and emotional skills (e.g., self-regulation) in determining parenting practices and behaviours and the importance of not only providing information but also supporting skills' development to change parenting behaviours (Bornstein 2015; Cuartas et al. 2022). Finally, these results align with quantitative empirical studies in Colombia, which found that cognitions, caregivers' own upbringing and

TABLE 4 | Integrating the perspectives of stakeholders to identify key risk factors (barriers to preventing) and protective (promoting) factors to prevent VAC.

	Risk factors and barriers to preventing VAC	Protective and promoting factors to prevent VAC	Key programme components
Micro Fathers, mothers and caregivers	• Emotional dysregulation • Lack of knowledge about child development and developmentally appropriate discipline • Attitudes and beliefs • Childhood adversities • Daily stressors • Little time available • Children's characteristics ◦ Age ◦ Behaviour • Negative perception of the ICBF and existing programmes • Poverty	• Life skills ◦ Emotional regulation ◦ Knowledge ◦ Empathy ◦ Organization skills ◦ Communication skills • Own upbringing without violence • Family composition • Attitudes and beliefs • Shared care load • Economic security	• Structure ◦ Clear and agreed theory of change ◦ User-friendly and relevant manuals and materials ◦ Monitoring, learning and evaluation systems ◦ Examples and cases that resonate with families ◦ Universal targeting and multiple caregivers ◦ Integration with existing systems and services • Contents ◦ Self-regulation techniques ◦ Communication techniques ◦ Needs of children at different stages of development ◦ Play and early stimulation ◦ Developmentally appropriate discipline ◦ Problem-solving ◦ Resilience ◦ Self-care and wellness ◦ Involving other caregivers in care • Delivery methods ◦ Positive feedback ◦ Demonstrations ◦ Role-play ◦ Practice of behaviours ◦ Group support ◦ Goal setting ◦ Joint problem-solving ◦ Homeworks ◦ Materials (print, toys) ◦ Innovative media (WhatsApp, chatbots)
Meso Facilitators/staff of prevention programmes	• Overloaded staff ◦ Inadequate training ◦ Attitudes that legitimize violence against children ◦ Work overload • Programme design ◦ Lack of monitoring and evaluation ◦ Programmes not relevant or not acceptable ◦ Rigidity of programmes ◦ Programmes not feasible ◦ Restrictive targeting (only some families, only some family members)	• Well-being and qualification of facilitators ◦ Adequate training ◦ Communication skills ◦ Emotion regulation skills ◦ Perspectives/attitudes towards violence ◦ Adequate workload • Programme design ◦ Monitoring and evaluation system ◦ Adaptation to cultural and local reality ◦ Relevant delivery formats ◦ Flexibility and innovation ◦ Articulation with existing infrastructure ◦ Family access ◦ Acceptability	
Macro Social structures and public policies	• Social structures ◦ Poverty ◦ Contextual violence ◦ Socio-economic inequality ◦ Patriarchal structures ◦ Discrimination and oppression • Public policies ◦ Disconnection within and between institutions ◦ Lack of theories of change ◦ Insufficient training programmes ◦ Waste of existing infrastructure	• Social structures ◦ Peace and security ◦ Socio-economic equity ◦ Social protection ◦ Gender equality ◦ Strong communities • Public politics ◦ Alliances and programmes for the prevention of VAC ◦ Shared vision of policymakers ◦ Coaching and training programmes • Use of existing infrastructure	

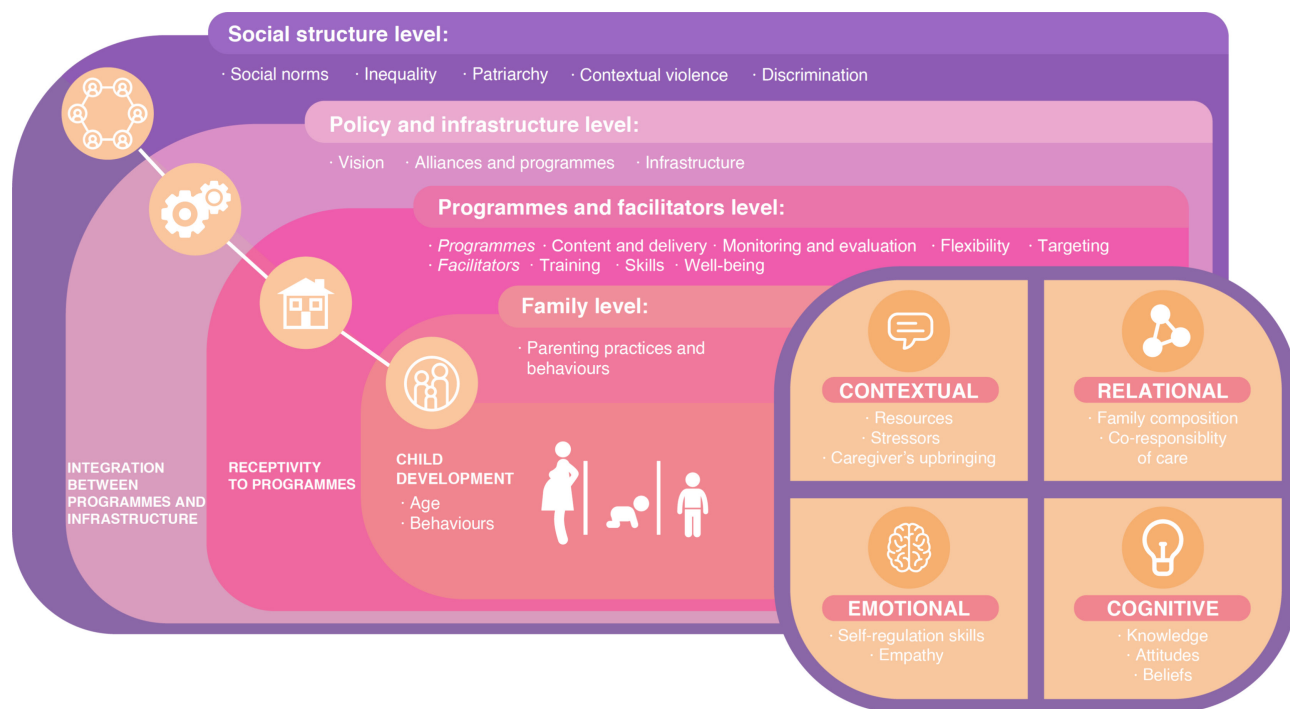


FIGURE 1 | An ecological, developmental and skills-based perspective on risk and protection factors.

contextual stressors predict the use of physical punishment (Cuartas et al. 2019).

At the meso level, Colombian stakeholders perceive facilitators' and programmes' characteristics as promoters/barriers to VAC prevention. On the one hand, programme facilitators' skills, training and work conditions are perceived as key factors to ensure programme implementation outcomes and effectiveness, consistent with evidence on the importance of facilitators' training and skills from a qualitative systematic review (Mytton et al. 2014). On the other hand, the acceptability, accessibility, cultural appropriateness, flexibility, effective delivery approaches, monitoring and evaluation, and articulation with existing infrastructure of programmes are also facilitators of VAC prevention. Prior research has reached similar conclusions, demonstrating the importance of adapting programmes to the cultural and local realities and using specific, effective delivery approaches (e.g., demonstrations and role-playing) to maximize implementation outcomes (e.g., Cuartas et al. 2022; Melendez-Torres, Leijten, and Gardner 2019; van der Put et al. 2018).

Finally, at the macro level, stakeholders stressed the importance of public policies and social structures in promoting or hindering VAC prevention. Some promoting factors related to public policies included a shared vision among policymakers, strengthening intersectoral work and employing existing infrastructure to embed innovations. Regarding social structures, stakeholders highlighted the role of social norms, inequality, patriarchal structures, discrimination and oppression, and contextual violence as facilitators or barriers to VAC prevention. These results are consistent with empirical findings on the role of the broader context in driving or inhibiting VAC (Cuartas et al. 2019; Ward et al. 2021) and demonstrate the importance

of the overall policy environment, and not only specific prevention programmes, in ensuring positive implementation outcomes and impact.

The qualitative design of this study enabled an in-depth examination of policymakers', facilitators' and caregivers' perspectives to understand critical aspects of the implementation ecosystem of VAC prevention programmes. Yet, future research should complement the current findings with quantitative data (e.g., data collected in randomized trials) to further explore social norms, beliefs and attitudes towards VAC and prevention programmes in Colombia. Moreover, more research is needed to assess whether the contents and delivery approaches that stakeholders consider as important are actually effective in Colombia, which could be explored using methods like factorial designs. In addition, the current sample included primarily mothers, so future research should complement our findings by exploring fathers' perspectives to inform the engagement of fathers in violence prevention parenting programmes. Finally, the current research focused on urban parents, so there is a need for further investigation of the perspectives of parents in rural areas and specific groups like Indigenous and ethnic communities.

Collectively, the current findings and prior local research (e.g., Cuartas et al. 2022) demonstrate the importance of ecological (i.e., the role of the broader ecology in increasing or mitigating the risk of VAC), developmental (i.e., the different needs and risks that families experience as children develop) and capability-based perspectives (i.e., the importance of caregivers' and facilitators' skills to prevent VAC) and provide insights on the whole 'implementation ecosystem' to inform the structure, content and delivery approaches of violence prevention programmes in Colombia (Table 4). Regarding structure, programmes should have a clear and common agreement theory of change, user-friendly and

relevant manuals and materials, monitoring and evaluation systems, universal targeting and focus on multiple caregivers and strong integration with existing systems and services. Regarding content, the programmes should target key drivers of VAC, including emotional self-regulation, communication, and problem-solving skills; knowledge on normative child development and developmentally appropriate discipline; the promotion of play; and caregivers' self-care and well-being. Finally, programmes should prioritize interactive and skills-based delivery approaches like positive feedback, demonstrations, role-play, practising behaviours, goal setting and joint problem-solving.

To conclude, this study provides evidence on the implementation ecosystem of violence prevention programmes in Colombia to inform the design of evidence-based novel strategies and programmes. Doing so is critical as VAC has increased amid the pandemic (Bhatia et al. 2021), and Colombia is facing a historical moment as recent peace treaties with armed groups opened a window of opportunity to invest in peace building and human development.

Author Contributions

Jorge Cuartas: conceptualization, investigation, writing – original draft, methodology, validation, visualization, writing – review and editing, software, formal analysis, project administration, supervision. **Luciana Beccassino:** conceptualization, investigation, methodology, writing – original draft, formal analysis, project administration. **Helen Baker-Henningham:** conceptualization, investigation, methodology, validation, supervision. **Oscar Iván Pineda Díaz:** conceptualization, investigation, methodology, validation. **María Fernanda Rodríguez:** conceptualization, investigation, methodology, validation.

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Conflicts of Interest

The authors declare no conflicts of interest.

Data Availability Statement

The data that support the findings of this study are available on request from the corresponding author. The data are not publicly available due to privacy or ethical restrictions.

References

- About, F. E., A. K. Yousafzai, and M. Nores. 2018. "State of the Science on Implementation Research in Early Child Development and Future Directions." *Annals of the new York Academy of Sciences* 1419, no. 1: 264–271. <https://doi.org/10.1111/nyas.13722>.
- Bhatia, A., C. Fabbri, I. Cerna-Turoff, et al. 2021. "Violence Against Children During the COVID-19 Pandemic." *Bulletin of the World Health Organization* 99, no. 10: 730–738. <https://doi.org/10.2471/BLT.20.283051>.
- Bornstein, M. H. 2015. "Children's Parents." In *Handbook of Child Psychology and Developmental Science*, edited by R. M. Lerner, M. H. Bornstein, and T. Leventhal, vol. 4, 55–132. John Wiley & Sons Inc. <https://doi.org/10.1002/9781118963418.childpsy403>.
- Bornstein, M. H., L. Cluver, K. Deater-Deckard, et al. 2022. "The Future of Parenting Programs: I Design." *Parenting* 22, no. 3: 201–234. <https://doi.org/10.1080/15295192.2022.2087040>.
- Braun, V., and V. Clarke. 2006. "Using Thematic Analysis in Psychology." *Qualitative Research in Psychology* 3, no. 2: 77–101. <https://doi.org/10.1191/1478088706qp0630a>.
- Britto, P. R., R. H. Bradley, H. Yoshikawa, L. A. Ponguta, L. Richter, and J. A. Kotler. 2022. "The Future of Parenting Programs: III Uptake and Scale." *Parenting* 22, no. 3: 258–275. <https://doi.org/10.1080/15295192.2022.2086809>.
- Bronfenbrenner, U., and P. Morris. 2007. "The Bioecological Model of Human Development." In *Handbook of Child Psychology*, edited by W. Damon, R. M. Lerner, and R. M. Lerner. <https://doi.org/10.1002/9780470147658.chpsy0114>.
- Chen, M., and K. L. Chan. 2015. "Effects of Parenting Programs on Child Maltreatment Prevention: A Meta-Analysis." *Trauma, Violence & Abuse* 17, no. 1: 88–104. <https://doi.org/10.1177/1524838014566718>.
- Coore Desai, C., J.-A. Reece, and S. Shakespeare-Pellington. 2017. "The Prevention of Violence in Childhood Through Parenting Programmes: A Global Review." *Psychology, Health & Medicine* 22, no. suppl: 166–186. <https://doi.org/10.1080/13548506.2016.1271952>.
- Cuartas, J. 2018. "Physical Punishment Against the Early Childhood in Colombia: National and Regional Prevalence, Sociodemographic Gaps, and Ten-Year Trends." *Children and Youth Services Review* 93: 428–440. <https://doi.org/10.1016/j.childyouth.2018.08.024>.
- Cuartas, J., A. Bhatia, D. Carter, et al. 2023. "The Climate Crisis and Violence Against Children." *Lancet Child & Adolescent Health* 7: 605–607. [https://doi.org/10.1016/S2352-4642\(23\)00137-2](https://doi.org/10.1016/S2352-4642(23)00137-2).
- Cuartas, J., A. Grogan-Kaylor, J. Ma, and B. Castillo. 2019. "Civil Conflict, Domestic Violence, and Poverty as Predictors of Corporal Punishment in Colombia." *Child Abuse & Neglect* 90: 108–119. <https://doi.org/10.1016/j.chiabu.2019.02.003>.
- Cuartas, J., H. Baker-Henningham, A. Cepeda, C. Rey-Guerra, and Instituto Colombiano de Bienestar Familiar (ICBF) Early Childhood Team. 2022. "The Apapacho Violence Prevention Parenting Program: Conceptual Foundations and Pathways to Scale." *International Journal of Environmental Research and Public Health* 19, no. 14: 8582. <https://www.mdpi.com/1660-4601/19/14/8582>.
- Draper, C. E., L. M. Barnett, C. J. Cook, et al. 2023. "Publishing Child Development Research From Around the World: An Unfair Playing Field Resulting in Most of the World's Child Population Under-Represented in Research." *Infant and Child Development* 32, no. 6: e2375. <https://doi.org/10.1002/icd.2375>.
- Gobierno de Colombia. 2021. "Estrategia nacional pedagógica y de prevención del castigo físico, tratos crueles, humillantes o degradantes contra niños, niñas y adolescentes".
- Guest, G., E. Namey, J. Taylor, N. Eley, and K. McKenna. 2017. "Comparing Focus Groups and Individual Interviews: Findings From a Randomized Study." *International Journal of Social Research*

- Methodology 20, no. 6: 693–708. <https://doi.org/10.1080/13645579.2017.1281601>.
- Hillis, S., J. Mercy, A. Amobi, and H. Kress. 2016. "Global Prevalence of Past-Year Violence Against Children: A Systematic Review and Minimum Estimates." *Pediatrics* 137, no. 3: e20154079. <https://doi.org/10.1542/peds.2015-4079>.
- Knerr, W., F. Gardner, and L. Cluver. 2013. "Improving Positive Parenting Skills and Reducing Harsh and Abusive Parenting in Low- and Middle-Income Countries: A Systematic Review." *Prevention Science* 14, no. 4: 352–363. <https://doi.org/10.1007/s11121-012-0314-1>.
- Lambert, S. D., and C. G. Loiselle. 2008. "Combining Individual Interviews and Focus Groups to Enhance Data Richness." *Journal of Advanced Nursing* 62, no. 2: 228–237. <https://doi.org/10.1111/j.1365-2648.2007.04559.x>.
- Lansford, J. E., T. S. Betancourt, K. Boller, et al. 2022. "The Future of Parenting Programs: II Implementation." *Parenting* 22, no. 3: 235–257. <https://doi.org/10.1080/15295192.2022.2086807>.
- Leijten, P., G. J. Melendez-Torres, W. Knerr, and F. Gardner. 2016. "Transported Versus Homegrown Parenting Interventions for Reducing Disruptive Child Behavior: A Multilevel Meta-Regression Study." *Journal of the American Academy of Child & Adolescent Psychiatry* 55, no. 7: 610–617. <https://doi.org/10.1016/j.jaac.2016.05.003>.
- McCoy, A., G. J. Melendez-Torres, and F. Gardner. 2020. "Parenting Interventions to Prevent Violence Against Children in Low- and Middle-Income Countries in East and Southeast Asia: A Systematic Review and Multi-Level Meta-Analysis." *Child Abuse & Neglect* 103: 104444. <https://doi.org/10.1016/j.chiabu.2020.104444>.
- McLaughlin, K. A., D. Weissman, and D. Bitrán. 2019. "Childhood Adversity and Neural Development: A Systematic Review." *Annual Review of Developmental Psychology* 1, no. 1: 277–312. <https://doi.org/10.1146/annurev-devpsych-121318-084950>.
- Melendez-Torres, G. J., P. Leijten, and F. Gardner. 2019. "What Are the Optimal Combinations of Parenting Intervention Components to Reduce Physical Child Abuse Recurrence? Reanalysis of a Systematic Review Using Qualitative Comparative Analysis." *Child Abuse Review* 28, no. 3: 181–197. <https://doi.org/10.1002/car.2561>.
- Mytton, J., J. Ingram, S. Manns, and J. Thomas. 2014. "Facilitators and Barriers to Engagement in Parenting Programs: A Qualitative Systematic Review." *Health Education & Behavior* 41, no. 2: 127–137. <https://doi.org/10.1177/1090198113485755>.
- Red Nacional de Información. 2018. "Registro Único de Víctimas (RUV)." <https://www.unidadvictimas.gov.co/es/registro-unico-de-victimas-ruv/37394>.
- Ritchie, J., and L. Spencer. 2002. "Qualitative Data Analysis for Applied Policy Research." In *The Qualitative Researcher's Companion*, edited by A. M. Huberman and M. B. Miles. SAGE Publications, Inc. <https://doi.org/10.4135/9781412986274>.
- Sameroff, A. 2010. "A Unified Theory of Development: A Dialectic Integration of Nature and Nurture." *Child Development* 81, no. 1: 6–22. <https://doi.org/10.1111/j.1467-8624.2009.01378.x>.
- Shenderovich, Y., J. M. Lachman, C. L. Ward, et al. 2021. "The Science of Scale for Violence Prevention: A New Agenda for Family Strengthening in Low- and Middle-Income Countries [Perspective]." *Frontiers in Public Health* 9: 581440. <https://doi.org/10.3389/fpubh.2021.581440>.
- Shonkoff, J. P., and A. S. Garner. 2011. "The Lifelong Effects of Early Childhood Adversity and Toxic Stress." *Pediatrics* 129: e232–e246. <https://doi.org/10.1542/peds.2011-2663>.
- Skar, A. S., L. Sherr, A. Macedo, S. V. Tetzchner, and K. I. Fostervold. 2021. "Evaluation of Parenting Interventions to Prevent Violence Against Children in Colombia: A Randomized Controlled Trial." *Journal of Interpersonal Violence* 36, no. 1–2: NP1098–NP1126. <https://doi.org/10.1177/0886260517736881>.
- UNICEF. 2020. "Designing Parenting Programmes for Violence Prevention: A Guidance Note." <https://www.unicef.org/documents/designing-parenting-programmes-violence-prevention-guidance-note>.
- van der Put, C. E., M. Assink, J. Gubbels, and N. F. Boekhout van Solinge. 2018. "Identifying Effective Components of Child Maltreatment Interventions: A Meta-Analysis." *Clinical Child and Family Psychology Review* 21, no. 2: 171–202. <https://doi.org/10.1007/s10567-017-0250-5>.
- Ward, K. P., A. Grogan-Kaylor, G. T. Pace, J. Cuartas, and S. J. Lee. 2021. "A Multilevel Ecological Analysis of the Predictors of Spanking Across 65 Countries." *BMJ Open* 11: e046075.
- World Health Organization. 2016. "INSPIRE: Seven Strategies for Ending Violence Against Children." <https://www.refworld.org/docid/57878f5e4.html>.